STATISTICAL ANALYSIS PLAN

A Phase 3 Randomised, Placebo-controlled Study of Kelvaparib as Maintenance Therapy in Recurrent High-grade Serous Ovarian Carcinoma

Study KLV-7701

Sponsor	Marnholt Oncology GmbH
Study number	KLV-7701
Phase	Phase 3
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Author	Biostatistics, Marnholt
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1. INTRODUCTION

This plan describes the analyses to be performed for study KLV-7701. It expands the statistical section of the protocol and takes precedence where the two differ. It was finalised and approved before the database was locked on 13 June 2022 and before any unblinded analysis.

2. OBJECTIVES AND ENDPOINTS

The primary objective is progression-free survival by investigator assessment. The primary endpoint is progression-free survival, defined as the time from the first dose of study treatment to the first documented disease progression by investigator assessment or death from any cause, whichever occurs first.

Secondary endpoints are duration of response, overall survival and the incidence and severity of treatment-emergent adverse events. Overall survival is defined as the time from the first dose of study treatment to death from any cause. Duration of response is defined, for a subject with a confirmed complete or partial response, as the time from the first dose of study treatment to the end of response recorded for the subject in the analysis dataset.

3. STUDY DESIGN

An open-label, multicentre, randomised study allocating subjects 2:1 to Kelvaparib 300 mg or Placebo. Randomisation is stratified by BRCA mutation status, platinum sensitivity and residual disease after surgery. Approximately 627 subjects.

4. ANALYSIS POPULATIONS

Population	Definition	Used for
Safety analysis set	All subjects who received at least one dose of study treatment.	All safety analyses and exposure.
Full analysis set	All subjects in the safety analysis set with at least one post-baseline disease assessment. Analysed as randomised.	All efficacy analyses.
Per-protocol set	Full analysis set subjects without a protocol deviation leading to discontinuation of study treatment.	Sensitivity analysis of the primary endpoint.
Pharmacokinetic analysis set	Subjects with at least one evaluable post-dose concentration.	Pharmacokinetic summaries.

5. GENERAL CONSIDERATIONS

Continuous variables are summarised by n, mean, standard deviation, median, first and third quartiles, minimum and maximum. Categorical variables by frequency and percentage with the denominator stated. Percentages are rounded to one decimal place and suppressed where the denominator is fewer than five. Analyses are performed using SAS version 9.4 or later.

Baseline is the last non-missing assessment on or before the first dose. A treatment-emergent adverse event is any event with onset on or after the first dose, or a pre-existing event that worsens after the first dose.

6. DISPOSITION AND BASELINE CHARACTERISTICS

Disposition is summarised by treatment group, giving the numbers screened, treated, ongoing and discontinued with reasons. Demographic and baseline disease characteristics are summarised descriptively; no formal comparison is performed.

7. EFFICACY ANALYSES

7.1 Primary endpoint

Progression-free survival is compared between groups using a log-rank test stratified by the randomisation factors at a two-sided significance level of 0.05. The hazard ratio and 95 per cent confidence interval are from a stratified Cox model with treatment as the only covariate. Medians with 95 per cent confidence limits are presented by group. The proportional hazards assumption is examined using scaled Schoenfeld residuals and the log-negative-log plot; if unsupported, a restricted mean survival time analysis is presented as supportive.

7.2 Secondary endpoints

Duration of response and overall survival are estimated by the Kaplan-Meier method with medians and 95 per cent confidence limits by the Brookmeyer-Crowley method. Binomial proportions are summarised with exact 95 per cent confidence intervals.

7.3 Multiplicity

The overall type I error rate is controlled at two-sided 0.05 by a fixed-sequence hierarchical testing procedure. The primary endpoint is tested first. If and only if it reaches significance are the key secondary endpoints tested, in this order, each at two-sided 0.05: first the objective response rate (Kelvaparib 300 mg versus placebo, Cochran-Mantel-Haenszel test stratified by the randomisation factors), then overall survival (stratified log-rank test). The full two-sided alpha of 0.05 passes to each endpoint in turn and is not split. Duration of response is descriptive. Testing stops at the first endpoint that does not reach significance and no endpoint below that point may be declared statistically significant; p-values below that point are reported as nominal and identified as such in every output.

7.4 Subgroup analyses

The primary endpoint is summarised within subgroups defined by BRCA mutation status, homologous recombination status, platinum sensitivity, residual disease after surgery, prior lines of platinum-based therapy, age category, ECOG performance status. Subgroup analyses are exploratory, are not adjusted for multiplicity, and no significance is claimed from them.

7.5 Sensitivity analyses

The primary analysis is repeated in the per-protocol set.

8. SAFETY ANALYSES

Treatment-emergent adverse events are summarised by system organ class and preferred term using MedDRA version 25.0, with subjects counted once per term at their highest grade. Separate summaries cover events of grade 3 or higher, serious events, related events, events leading to dose modification or discontinuation, and fatal events. Adverse events of special interest (myelodysplastic syndrome or acute myeloid leukaemia, pneumonitis, severe anaemia) are summarised individually with time to onset and resolution.

Laboratory data are summarised by shift from baseline to worst post-baseline CTCAE grade. Vital signs are summarised by change from baseline.

9. PHARMACOKINETIC ANALYSES

Concentrations are summarised by nominal sampling time using n, arithmetic mean, standard deviation, geometric mean, geometric coefficient of variation, median, minimum and maximum. Derived parameters are summarised for the kelvaparib 300 mg group. Concentrations below the limit of quantification are set to zero for descriptive summaries and excluded from geometric means.

10. INTERIM ANALYSES

A non-binding futility interim analysis is performed when approximately half of the planned subjects have completed the primary endpoint assessment. No alpha is spent.

11. CHANGES FROM THE PROTOCOL-SPECIFIED ANALYSES

This plan clarified which categories of protocol deviation are considered important for the per-protocol set and extended the subgroup list. Neither change affects the primary analysis. No change was made after database lock.

12. TABLE, LISTING AND FIGURE SPECIFICATIONS

The index of outputs and their shells is provided in the appendix. Numbering follows ICH E3: Section 14.1 demographic and disposition, 14.2 efficacy (including the subgroup analyses of the primary endpoint), 14.3 safety, 14.4 pharmacokinetics.

13 STUDY OBJECTIVES AND ENDPOINTS

The objectives and endpoints analysed under this plan are those in the protocol. Where this plan gives a more precise definition of an endpoint than the protocol, this plan governs the analysis and the difference is recorded in Section 19.

14 GENERAL ANALYSIS CONVENTIONS

14.1 Definition of baseline

Baseline is the last non-missing assessment obtained on or before the date and time of the first dose of study treatment. Where an assessment on the day of first dose has no time recorded it is taken as pre-dose unless another record shows otherwise.

14.2 Definition of study day

Study day is calculated relative to the date of first dose, which is day 1. There is no day 0; the day before first dose is day -1. This convention is applied in every dataset and every output.

14.3 Treatment-emergent period

An event is treatment-emergent if its onset is on or after the first dose, or if it was present before the first dose and worsened after it. Adverse events are collected to the end of study participation (protocol Section 7.3).

14.4 Handling of partial and missing dates

A partial date is completed conservatively: a missing day is set to the first of the month for a start date and the last of the month for an end date; a missing month is set to January for a start date and December for an end date. Where completion would place a start date before the first dose, the event is treated as treatment-emergent unless other information shows otherwise.

14.5 Presentation conventions

Continuous variables are summarised by the number of observations, mean, standard deviation, median, minimum and maximum. Percentages are presented to one decimal place and calculated on the number of subjects in the analysis set unless a different denominator is stated in the table. Confidence intervals are two-sided at the 95 per cent level unless stated otherwise.

15 ANALYSIS SETS

Table 1 Analysis sets

Set	Definition	Used for
Enrolled	All subjects assigned to treatment.	Disposition and deviations.
Safety	All subjects who received at least one dose, analysed as treated.	All safety analyses.
Full analysis	All subjects in the safety analysis set with at least one post-baseline disease assessment, analysed as randomised.	Primary and secondary efficacy.
Per protocol	Full analysis set subjects without a protocol deviation leading to discontinuation of study treatment.	Sensitivity analysis of the primary endpoint.
Efficacy evaluable	The same subjects as the full analysis set.	Response analyses.
Pharmacokinetic	Subjects with at least one measurable post-dose concentration and no deviation affecting it.	Pharmacokinetic summaries.

16 ESTIMANDS

The estimand for the primary endpoint is defined by its population, variable, the handling of intercurrent events and the population-level summary. The population is the full analysis set: every treated subject with at least one post-baseline disease assessment, analysed as randomised. The variable is progression-free survival as defined in Section 2. Intercurrent events: discontinuation of study treatment for any reason, and the start of a subsequent anti-disease therapy, are handled by the treatment policy strategy, so follow-up and data after either continue to contribute; death is not an intercurrent event but part of the variable (it is an event). The population-level summary is the hazard ratio (Kelvaparib 300 mg versus placebo) from a Cox model stratified by the randomisation factors, with its 95 per cent confidence interval, and the treatment groups are compared by the log-rank test stratified by the same factors.

17 STATISTICAL METHODS

17.1 Primary analysis

Progression-free survival, defined as the time from the first dose of study treatment to the first documented disease progression by investigator assessment or death from any cause, whichever occurs first.

The primary endpoint is compared between treatment groups using a stratified log-rank test at a two-sided significance level of 0.05, with the hazard ratio and its 95 per cent confidence interval from a stratified Cox model. Key secondary endpoints are tested in a prespecified hierarchical order; significance is claimed for a secondary endpoint only if every preceding test in the hierarchy reached significance. Approximately 388 events provide 90 per cent power to detect a hazard ratio of 0.71.

17.2 Censoring rules

Progression-free survival: a subject with neither documented progression nor death is censored at the date of the last assessment at which absence of progression was documented or, if still in follow-up at the data cut-off, at the data cut-off date; a subject with no post-baseline assessment is censored at day 1; death without documented progression is an event at the date of death. Overall survival: a subject not known to have died is censored at the date last known alive or, if still in follow-up at the data cut-off, at the data cut-off date. Duration of response: measured from the first dose of study treatment to the recorded end of response; the response and progression dates are as held in the analysis datasets, which hold no assessment-level tumour data from which they could be re-derived; a responder whose response has not ended is censored at the date of the last assessment or, if still in follow-up at the data cut-off, at the data cut-off date. No subject is censored at the start of a subsequent anti-disease therapy.

17.3 Sensitivity and supplementary analyses

- Repeat of the primary analysis on the per-protocol set.
- Repeat of the primary analysis treating subjects with no post-baseline assessment as non-responders.
- Unstratified repeat of any stratified analysis, to show the contribution of the stratification.

17.4 Subgroup analyses

The primary endpoint is examined in the prespecified subgroups listed in the table shells. These analyses are descriptive, are not adjusted for multiplicity, and are not used to claim an effect in any subgroup.

17.5 Multiplicity

The overall type I error rate is controlled at two-sided 0.05 by a fixed-sequence hierarchical testing procedure. The primary endpoint is tested first. If and only if it reaches significance are the key secondary endpoints tested, in this order, each at two-sided 0.05: first the objective response rate (Kelvaparib 300 mg versus placebo, Cochran-Mantel-Haenszel test stratified by the randomisation factors), then overall survival (stratified log-rank test). The full two-sided alpha of 0.05 passes to each endpoint in turn and is not split. Duration of response is descriptive. Testing stops at the first endpoint that does not reach significance and no endpoint below that point may be declared statistically significant; p-values below that point are reported as nominal and identified as such in every output.

17.6 Interim analyses

Any interim analysis is described here before it is performed, including its timing, the quantities to be examined, the alpha spent if any, and the actions available to the monitoring committee. The only interim analysis is the non-binding futility analysis described in Section 10, at which no alpha is spent.

17.7 Safety analyses

Safety analyses are descriptive and are performed on the safety set. Adverse events are coded with the Medical Dictionary for Regulatory Activities and graded with the Common Terminology Criteria for Adverse Events. A subject with more than one occurrence of the same preferred term is counted once at the highest grade. Laboratory results are graded and presented as shifts from baseline to the worst post-baseline grade.

17.8 Pharmacokinetic analyses

Concentrations below the lower limit of quantification are set to zero before the first measurable concentration and treated as missing thereafter. Parameters are summarised by geometric mean and geometric coefficient of variation. No formal comparison between doses is performed unless stated in the protocol.

18 TABLE, LISTING AND FIGURE INVENTORY

The outputs below are produced under this plan. Each is specified by its population, its content and its sort order. Shells are held in the output specification document.

Table 2 Planned tables and figures

Number	Title	Population	Content
14.1.1	Demographic and Baseline Characteristics	Safety	Descriptive statistics by treatment group and overall.
14.1.2	Subject Disposition	Safety	Number and percentage by disposition category and treatment group.
14.1.3	Extent of Exposure	Safety	Duration of treatment, cycles started, relative dose intensity, dose reductions and interruptions.
14.2.1	Best Overall Response by RECIST 1.1	Full analysis	Best overall response by RECIST 1.1 and objective response rate with its exact 95 per cent confidence interval.
14.2.2	Kaplan-Meier Estimates of Time-to-event Endpoints	Full analysis	Kaplan-Meier median and quartiles of duration of response, progression-free survival and overall survival.
14.2.3	Overall Response Rate and Progression-free Survival by BRCA Mutation Status	Full analysis	Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.
14.2.4	Overall Response Rate and Progression-free Survival by Homologous Recombination Status	Full analysis	Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.
14.2.5	Overall Response Rate and Progression-free Survival by Platinum Sensitivity	Full analysis	Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.

Number	Title	Population	Content
14.2.6	Overall Response Rate and Progression-free Survival by Residual Disease After Surgery	Full analysis	Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.
14.2.7	Overall Response Rate and Progression-free Survival by Prior Lines of Platinum-based Therapy	Full analysis	Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.
14.2.8	Overall Response Rate and Progression-free Survival by Age Category	Full analysis	Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.
14.2.9	Overall Response Rate and Progression-free Survival by ECOG Performance Status	Full analysis	Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.
14.3.1	Overview of Treatment-emergent Adverse Events	Safety	Number and percentage with any event, grade 3 or higher, serious, fatal, leading to dose modification or discontinuation.
14.3.2	Treatment-emergent Adverse Events Occurring in at Least 2% of Subjects by System Organ Class and Preferred Term	Safety	Number and percentage by system organ class and preferred term, sorted by descending frequency.
14.3.3	Treatment-emergent Adverse Events of Grade 3 or Higher by System Organ Class and Preferred Term	Safety	As the table by system organ class and preferred term, restricted to grade 3 or higher.
14.3.4	Treatment-related Treatment-emergent Adverse Events Occurring in at Least 2% of Subjects	Safety	Number and percentage of events assessed as related, by preferred term.

Number	Title	Population	Content
14.3.5	Serious Treatment-emergent Adverse Events by Preferred Term	Safety	Number and percentage by preferred term.
14.3.6	Treatment-emergent Adverse Events Leading to Discontinuation of Study Treatment	Safety	Number and percentage by preferred term.
14.3.7	Deaths	Safety	Number and percentage by primary cause.
14.3.8	Shift from Baseline to Worst Post-baseline CTCAE Grade for Selected Laboratory Parameters	Safety	Worst post-baseline CTCAE grade and shifts of two or more grades from baseline, by parameter.
14.3.9	Time to Onset and Resolution of the Most Frequent Treatment-emergent Adverse Events	Safety	Median time to onset and to resolution of the most frequent events.
14.3.10	Overview of adverse events by BRCA mutation status	Safety	Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.
14.3.11	Overview of adverse events by homologous recombination status	Safety	Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.
14.3.12	Overview of adverse events by platinum sensitivity	Safety	Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.
14.3.13	Overview of adverse events by residual disease after surgery	Safety	Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.

Number	Title	Population	Content
14.3.14	Overview of adverse events by prior lines of platinum-based therapy	Safety	Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.
14.3.15	Overview of adverse events by age category	Safety	Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.
14.3.16	Overview of adverse events by ECOG performance status	Safety	Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.
14.4.1	Summary of Steady-state Pharmacokinetic Parameters	Pharmacokinetic	Summary statistics of steady-state pharmacokinetic parameters.
14.4.2	Accumulation Ratio and Trough Concentration	Pharmacokinetic	Accumulation ratio and trough concentration.

18.1 Shells

A shell is given for every output. The shell fixes the title, the column structure, the denominators, the sort order and the footnotes, so that the output can be programmed and reviewed before any data are seen.

Table 14.1.1 Demographic and Baseline Characteristics

Population: Safety analysis set

Content: Descriptive statistics by treatment group and overall.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.1.2 Subject Disposition

Population: Safety analysis set

Content: Number and percentage by disposition category and treatment group.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.1.3 Extent of Exposure

Population: Safety analysis set

Content: Duration of treatment, cycles started, relative dose intensity, dose reductions and interruptions.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.1 Best Overall Response by RECIST 1.1

Population: Full analysis analysis set

Content: Best overall response by RECIST 1.1 and objective response rate with its exact 95 per cent confidence interval.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.2 Kaplan-Meier Estimates of Time-to-event Endpoints

Population: Full analysis analysis set

Content: Kaplan-Meier median and quartiles of duration of response, progression-free survival and overall survival.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.3 Overall Response Rate and Progression-free Survival by BRCA Mutation Status

Population: Full analysis analysis set

Content: Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.

Subgroup	n	Responders, n (%)	95% CI	Median PFS (months)
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.4 Overall Response Rate and Progression-free Survival by Homologous Recombination Status

Population: Full analysis analysis set

Content: Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.

Subgroup	n	Responders, n (%)	95% CI	Median PFS (months)
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.5 Overall Response Rate and Progression-free Survival by Platinum Sensitivity

Population: Full analysis analysis set

Content: Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.

Subgroup	n	Responders, n (%)	95% CI	Median PFS (months)
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.6 Overall Response Rate and Progression-free Survival by Residual Disease After Surgery

Population: Full analysis analysis set

Content: Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.

Subgroup	n	Responders, n (%)	95% CI	Median PFS (months)
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.7 Overall Response Rate and Progression-free Survival by Prior Lines of Platinum-based Therapy

Population: Full analysis analysis set

Content: Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.

Subgroup	n	Responders, n (%)	95% CI	Median PFS (months)
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.8 Overall Response Rate and Progression-free Survival by Age Category

Population: Full analysis analysis set

Content: Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.

Subgroup	n	Responders, n (%)	95% CI	Median PFS (months)
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.2.9 Overall Response Rate and Progression-free Survival by ECOG Performance Status

Population: Full analysis analysis set

Content: Objective response rate with its exact 95 per cent confidence interval and median progression-free survival in each subgroup level.

Subgroup	n	Responders, n (%)	95% CI	Median PFS (months)
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.1 Overview of Treatment-emergent Adverse Events

Population: Safety analysis set

Content: Number and percentage with any event, grade 3 or higher, serious, fatal, leading to dose modification or discontinuation.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.2 Treatment-emergent Adverse Events Occurring in at Least 2% of Subjects by System Organ Class and Preferred Term

Population: Safety analysis set

Content: Number and percentage by system organ class and preferred term, sorted by descending frequency.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.3 Treatment-emergent Adverse Events of Grade 3 or Higher by System Organ Class and Preferred Term

Population: Safety analysis set

Content: As the table by system organ class and preferred term, restricted to grade 3 or higher.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.4 Treatment-related Treatment-emergent Adverse Events Occurring in at Least 2% of Subjects

Population: Safety analysis set

Content: Number and percentage of events assessed as related, by preferred term.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.5 Serious Treatment-emergent Adverse Events by Preferred Term

Population: Safety analysis set

Content: Number and percentage by preferred term.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.6 Treatment-emergent Adverse Events Leading to Discontinuation of Study Treatment

Population: Safety analysis set

Content: Number and percentage by preferred term.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.7 Deaths

Population: Safety analysis set

Content: Number and percentage by primary cause.

Category	Kelvaparib 300 mg	Placebo	Total
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.8 Shift from Baseline to Worst Post-baseline CTCAE Grade for Selected Laboratory Parameters

Population: Safety analysis set

Content: Worst post-baseline CTCAE grade and shifts of two or more grades from baseline, by parameter.

Baseline grade	Post-baseline grade 0 to 1	Grade 2	Grade 3 or higher
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.9 Time to Onset and Resolution of the Most Frequent Treatment-emergent Adverse Events

Population: Safety analysis set

Content: Median time to onset and to resolution of the most frequent events.

Preferred term	Events, n	Median time to onset (days)	Resolved, n (%)	Median time to resolution (days)
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.10 Overview of adverse events by BRCA mutation status

Population: Safety analysis set

Content: Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.

Category	Subgroup level 1	Subgroup level 2	Subgroup level 3
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.11 Overview of adverse events by homologous recombination status

Population: Safety analysis set

Content: Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.

Category	Subgroup level 1	Subgroup level 2	Subgroup level 3
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.12 Overview of adverse events by platinum sensitivity

Population: Safety analysis set

Content: Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.

Category	Subgroup level 1	Subgroup level 2	Subgroup level 3
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.13 Overview of adverse events by residual disease after surgery

Population: Safety analysis set

Content: Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.

Category	Subgroup level 1	Subgroup level 2	Subgroup level 3
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.14 Overview of adverse events by prior lines of platinum-based therapy

Population: Safety analysis set

Content: Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.

Category	Subgroup level 1	Subgroup level 2	Subgroup level 3
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.15 Overview of adverse events by age category

Population: Safety analysis set

Content: Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.

Category	Subgroup level 1	Subgroup level 2	Subgroup level 3
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.3.16 Overview of adverse events by ECOG performance status

Population: Safety analysis set

Content: Subjects with any event, grade 3 or higher, serious, leading to discontinuation and fatal, in each subgroup level.

Category	Subgroup level 1	Subgroup level 2	Subgroup level 3
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.4.1 Summary of Steady-state Pharmacokinetic Parameters

Population: Pharmacokinetic analysis set

Content: Summary statistics of steady-state pharmacokinetic parameters.

Parameter	n	Geometric mean (CV%)	Range
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

Table 14.4.2 Accumulation Ratio and Trough Concentration

Population: Pharmacokinetic analysis set

Content: Accumulation ratio and trough concentration.

Parameter	n	Geometric mean (CV%)	Range
Row label 1	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 2	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 3	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 4	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 5	xx (xx.x)	xx (xx.x)	xx (xx.x)
Row label 6	xx (xx.x)	xx (xx.x)	xx (xx.x)

xx denotes a count and xx.x a percentage, to be replaced at production. Percentages are calculated on the number of subjects in the analysis set unless stated otherwise. Sort order: descending frequency in the total column, then alphabetically.

19 CHANGES FROM THE PROTOCOL

Where this plan differs from the analysis described in the protocol, the difference and the reason for it are recorded here. Any change made after database lock is reported in the clinical study report and identified as post hoc.

20 PROGRAMMING CONSIDERATIONS

Analyses are produced from the analysis datasets described in the dataset specification, which follow the Analysis Data Model. Outputs are produced by validated programs, and every output is independently reproduced from the same analysis datasets by a second programmer before it is released.